This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practise under it.

Healthcare professionals must work within their own competence. This PGD does not remove professional obligations or accountability.

ISSUED, VALID FROM 30 SEPTEMBER 2026

Patient Group Direction

for the supply of Calcipotriol with Betamethasone Dipropionate for the treatment of Plaque Psoriasis

Plaque Psoriasis

Adults aged 18 years and over.

Patient Group Direction, version 008, issued 30 September 2026. Calcipotriol 50 micrograms/g with betamethasone 0.5 mg/g, as ointment, gel, cream or cutaneous foam. Adults 18 and over, stable plaque psoriasis.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

What this PGD is for

The supply of a combined topical treatment to an adult with a flare of stable plaque psoriasis on the trunk, limbs or scalp, by a registered pharmacist or registered pharmacy technician in a community pharmacy, without a prescription.

What it authorises

- Calcipotriol 50 micrograms/g with betamethasone (as dipropionate) 0.5 mg/g, as ointment, gel, cream or cutaneous foam, once daily for up to 4 weeks, in adults aged 18 years and over.

What it does not cover

- The face, flexures or genital skin, where a potent steroid is not used.
- Generalised pustular psoriasis or erythroderma. Refer as an emergency.
- Psoriasis affecting more than 10% of the body surface (NICE's threshold for extensive disease), or needing more than 15g of product in a day. Refer.
- Anyone under 18.

Summary of NICE guidance for plaque psoriasis

Psoriasis: assessment and management, NICE clinical guideline CG153, published 24 October 2012, last updated 1 September 2017, with the Summaries of Product Characteristics for Dovobet 50 micrograms/g + 0.5 mg/g ointment and Enstilar 50 micrograms/g + 0.5 mg/g cutaneous foam (LEO Laboratories Limited, both revised 6 September 2024). Summarised 9 September 2026.

What plaque psoriasis is, and how it is assessed

Psoriasis is an inflammatory skin disease that typically follows a relapsing and remitting course. Plaque psoriasis, well delineated red scaly plaques varying from a few patches to generalised involvement, is by far the most common form and accounts for about 90% of people with psoriasis.

UK prevalence is estimated at around 1.3% to 2.2%. Most cases begin before the age of 35, and distinctive nail changes occur in around 50% of those affected.

For any type of psoriasis, assess disease severity, the impact on physical, psychological and social wellbeing, whether the person has psoriatic arthritis, and the presence of comorbidities. Assess at first presentation, before referral, at each referral point, and to evaluate how well treatment is working.

Record a static physician global assessment (clear, nearly clear, mild, moderate, severe or very severe), the patient own assessment of current severity, the body surface area affected, any involvement of nails or of high impact and difficult to treat sites (face, scalp, palms, soles, flexures and genitals), and any systemic upset such as fever and malaise, which is common in unstable forms such as erythroderma or generalised pustular psoriasis.

Assess impact by asking what aspects of daily living are affected, how the person is coping with the condition and the treatments, whether it affects their mood or causes distress, whether it affects family or carers, and whether they need further advice or support.

ERYTHEMA MAY BE UNDERESTIMATED IN DARKER SKIN, for example Fitzpatrick types 5 and 6.

Relapse occurs in most people after treatment is stopped. Both SmPCs describe once daily flare treatment for a recommended period of 4 weeks, with treatment continued or restarted after that period only following medical review and under regular medical supervision.

First-line topical treatment of the trunk and limbs

Offer topical treatment as first-line treatment, and offer the person a supply to keep at home for self-management.

For an adult with trunk or limb psoriasis, the initial treatment NICE recommends is a POTENT CORTICOSTEROID ONCE DAILY PLUS VITAMIN D OR A VITAMIN D ANALOGUE ONCE DAILY, applied separately, one in the morning and the other in the evening, FOR UP TO 4 WEEKS.

NICE offers a COMBINED PRODUCT containing calcipotriol monohydrate and betamethasone dipropionate once daily for up to 4 weeks WHEN a twice daily potent corticosteroid or a coal tar preparation cannot be used, OR when a once daily preparation would improve adherence. That is the product this PGD authorises.

Dovobet ointment is licensed for topical treatment of stable plaque psoriasis vulgaris amenable to topical therapy in adults, applied once daily, recommended treatment period 4 weeks.

Enstilar cutaneous foam is licensed for topical treatment of psoriasis vulgaris in adults, applied once daily, recommended treatment period 4 weeks. Shake the can and hold it at least 3 cm from the skin, in any orientation except horizontally.

MAXIMUM 15 g A DAY of calcipotriol containing products in total, across all such products the person is using. THE TOTAL BODY SURFACE AREA TREATED SHOULD NOT EXCEED 30% (the licence limit). THIS PGD APPLIES A LOWER CEILING OF 10% OF BODY SURFACE; ABOVE THAT, REFER.

Choose the formulation on patient preference, cosmetic acceptability, practical aspects of application, and the site and extent of disease: cream, lotion or gel for widespread psoriasis, lotion, solution or gel for the scalp or hair-bearing areas, and ointment for areas with thick adherent scale.

ARRANGE A REVIEW 4 WEEKS after starting a new topical treatment in an adult, to evaluate tolerability, toxicity and initial response, to reinforce adherence, and to reinforce the importance of a 4 week break between courses of potent or very potent corticosteroids.

Topical treatment alone may not give satisfactory control, especially where psoriasis is extensive (for example more than 10% of body surface area) or is at least moderate on the static physician global assessment. NICE's 10% figure is a threshold for considering specialist referral, and it is the ceiling applied in this PGD: psoriasis affecting more than 10% of body surface is referred, not supplied. The calcipotriol licence limit of 30% is not the PGD ceiling.

If there is no improvement

If there is little or no improvement at the 4 week review, discuss the next treatment option. Before changing treatment, discuss difficulties with application, cosmetic acceptability or tolerability, offer an alternative formulation where relevant, and consider other reasons for non-adherence.

If once daily potent corticosteroid plus once daily vitamin D or analogue does not achieve clearance, near clearance or satisfactory control of trunk or limb psoriasis in an adult after A MAXIMUM OF 8 WEEKS, offer vitamin D or a vitamin D analogue alone twice daily.

If twice daily vitamin D or analogue does not achieve clearance, near clearance or satisfactory control after 8 to 12 weeks, offer either a potent corticosteroid twice daily for up to 4 weeks, or a coal tar preparation once or twice daily.

Very potent corticosteroids for trunk or limb psoriasis in adults are for SPECIALIST SETTINGS ONLY, under careful supervision, when other topical strategies have failed, and for a maximum of 4 weeks.

Offer second-line or third-line options such as phototherapy or systemic treatment at the same time where topical treatment alone is unlikely to control the disease, for example extensive disease, disease at least moderate on the static physician global assessment, or nail disease.

Scalp, face, flexures and genitals, and why the combined product is not for those sites

For SCALP psoriasis the initial treatment is a potent corticosteroid once daily for up to 4 weeks, and the person should be shown how to apply it safely.

If that does not achieve clearance, near clearance or satisfactory control after 4 weeks, consider a different formulation of the potent corticosteroid such as a shampoo or mousse, and topical treatments to remove adherent scale such as salicylic acid agents, emollients and oils before applying the corticosteroid. Only if the response is still unsatisfactory after a further 4 weeks does NICE offer the combined calcipotriol with betamethasone product once daily for up to 4 weeks.

The Enstilar SmPC gives scalp instructions: spray into the palm of the hand and apply to affected scalp areas with the fingertips. The Dovobet ointment SmPC states there is limited experience of its use on the scalp.

For psoriasis of the FACE, FLEXURES OR GENITALS, offer a SHORT-TERM MILD OR MODERATE potency corticosteroid once or twice daily for a MAXIMUM OF 2 WEEKS.

DO NOT USE POTENT OR VERY POTENT CORTICOSTEROIDS ON THE FACE, FLEXURES OR GENITALS. These sites are particularly vulnerable to steroid atrophy, and corticosteroids should be used there only short-term, 1 to 2 weeks per month, with the risks explained.

THE COMBINED PRODUCT IS NOT FOR THESE SITES because it contains a potent group III steroid. Both SmPCs state that the skin of the face and genitals is very sensitive to corticosteroids and that the product should not be used in these areas, and that application on large areas of damaged skin, on mucous membranes or in skin folds should be avoided because it increases systemic absorption.

Avoid application under occlusive dressings, avoid concurrent treatment with other steroids on the same treatment area, and instruct the patient in correct use to avoid application and accidental transfer to the face, mouth and eyes.

Topical treatments at facial, flexural and genital sites may cause irritation. Tell the person about this and how to minimise it.

Limits on continuous potent corticosteroid use

DO NOT USE POTENT CORTICOSTEROIDS CONTINUOUSLY AT ANY SITE FOR LONGER THAN 8 WEEKS.

Do not use very potent corticosteroids continuously at any site for longer than 4 weeks, and do not use very potent corticosteroids in children and young people.

AIM FOR A BREAK OF 4 WEEKS BETWEEN COURSES of potent or very potent corticosteroids, and consider non-steroid topical treatments such as vitamin D or analogues, or coal tar, to maintain control during that break.

Continuous use of potent or very potent corticosteroids may cause irreversible skin atrophy and striae, may make psoriasis unstable, and may cause systemic side effects when applied continuously to extensive psoriasis, for example more than 10% of body surface area.

Offer a review at least annually to an adult using intermittent or short-term courses of a potent or very potent corticosteroid, as monotherapy or in a combined preparation, to assess for steroid atrophy and other adverse effects.

Both SmPCs state that long-term use increases the risk of local and systemic adverse reactions and that treatment should be discontinued if such reactions occur.

Local reactions after topical betamethasone dipropionate, especially with prolonged application, include skin atrophy, telangiectasia, striae, folliculitis, hypertrichosis, perioral dermatitis, allergic contact dermatitis, depigmentation and colloid milia.

Systemic reactions from topical corticosteroids are rare in adults but can be severe: adrenocortical suppression, cataract, infections, impaired glycaemic control in diabetes and raised intra-ocular pressure, particularly after long-term treatment.

The safety and efficacy of Dovobet ointment and Enstilar foam in people below 18 years have not been established.

When to refer, and the two emergencies

GENERALISED PUSTULAR PSORIASIS AND ERYTHRODERMA: refer IMMEDIATELY for same-day specialist assessment and treatment.

The combined product is CONTRAINDICATED in erythrodermic and pustular psoriasis, and Dovobet ointment is also contraindicated in exfoliative psoriasis.

Refer for dermatology specialist advice where there is diagnostic uncertainty, where any type of psoriasis is severe or extensive (for example more than 10% of body surface area), where it cannot be controlled with topical treatment, where nail disease has a major functional or cosmetic impact, or where the psoriasis is having a major impact on physical, psychological or social wellbeing.

REFER CHILDREN AND YOUNG PEOPLE with any type of psoriasis to a specialist at presentation.

REBOUND. When treating psoriasis with topical corticosteroids there may be a risk of generalised pustular psoriasis, or of rebound effects when discontinuing treatment, so medical supervision should continue in the post-treatment period. Rebound effect is an uncommon adverse reaction for Enstilar foam and a rare one for Dovobet ointment.

A reported case of misuse: a patient with extensive erythrodermic psoriasis used 240 g of Dovobet ointment weekly for 5 months, against a maximum recommended 15 g daily, developed Cushing syndrome during treatment, and then developed pustular psoriasis after abruptly stopping. In chronic toxicity, corticosteroid treatment must be discontinued gradually.

Refer or offer narrowband UVB phototherapy where plaque psoriasis cannot be controlled with topical treatments alone.

If lesions become secondarily infected, treat with antimicrobial therapy, and if the infection worsens, stop the corticosteroid.

Refer for evaluation by an ophthalmologist if the person presents with blurred vision or other visual disturbance. These have been reported with systemic and topical corticosteroid use, and possible causes include cataract, glaucoma and central serous chorioretinopathy.

What to tell the patient

Give information tailored to the person so they understand the diagnosis and treatment options, relevant lifestyle risk factors, when and how to treat, how to use the treatment safely and effectively including how to apply it, when and how to seek further review, and strategies to deal with the impact on wellbeing.

Explain the importance of continuing treatment until a satisfactory outcome is achieved, for example clear or nearly clear, or up to the maximum recommended treatment period for corticosteroids.

Explain that RELAPSE OCCURS IN MOST PEOPLE after treatment stops, and that after the initial treatment period topical treatments can be used when needed to maintain satisfactory control.

Explain the risks of skin atrophy, striae, unstable psoriasis and systemic side effects from continuous potent corticosteroid use, and how to avoid them.

Tell the person not to exceed 15 g a day in total from all calcipotriol containing products, not to treat more than 10% of the body surface area (the ceiling applied under this PGD; the licence limit is 30%), and not to use occlusive dressings.

Tell the person to WASH THEIR HANDS after each application, unless the hands are being treated, to avoid spreading the product to other parts of the body and unintended absorption through the hands.

Tell the person it is not recommended to shower or bathe immediately after application, and for Enstilar to let the foam remain on the skin or scalp during the night or the day.

Advise the person to limit or avoid excessive exposure to natural or artificial sunlight during treatment.

ENSTILAR FOAM IS AN EXTREMELY FLAMMABLE AEROSOL. The pressurised container may burst if heated. Protect from sunlight, do not expose to temperatures above 50C, do not pierce or burn even after use, do not spray on an open flame or other ignition source, keep away from sparks, open flames and other ignition sources, and do not smoke.

Both products contain butylhydroxytoluene (E321), which may cause local skin reactions such as contact dermatitis, or irritation to the eyes and mucous membranes.

Tell the person the label will state strong steroid.

Ask the person to report any suspected side effects, and report suspected adverse reactions through the MHRA Yellow Card Scheme.

The Patient Group Direction

How to use this PGD

This is stable plaque psoriasis only

- The licensed indication is stable plaque psoriasis vulgaris amenable to topical therapy, in adults.
- There is no experience with this product in GUTTATE psoriasis. Refer.
- Erythrodermic, exfoliative and pustular psoriasis are contraindicated and are emergencies. Refer the same day. See Appendix 1 for how to recognise them.
- Not for the face, genitals or flexures. The skin there is very sensitive to corticosteroids and the SPC says the product should not be used on those sites.

The numbers

- Once daily to affected areas.
- MAXIMUM 15g IN ANY ONE DAY. This is the SPC limit and it is what protects against hypercalcaemia.
- MAXIMUM 10% OF BODY SURFACE affected (NICE's threshold for extensive disease). Above that, refer; do not supply and advise partial use. The calcipotriol licence limit of 30% is not the ceiling for this PGD.
- Up to 100g supplied per week, within the 15g daily limit.
- FOUR WEEKS, then stop and review. Continuing or restarting beyond 4 weeks requires medical review and ongoing medical supervision, which is a GP decision and not a PGD one.

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Patient Group Direction

for the supply, for the treatment of Plaque Psoriasis, of:

Calcipotriol 50 micrograms/g with betamethasone 0.5 mg/g

Patient Group Direction for the supply of calcipotriol 50 micrograms/g with betamethasone (as dipropionate) 0.5 mg/g, as ointment, gel, cream or cutaneous foam, for stable plaque psoriasis in adults aged 18 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be familiar with the current SPC for the formulation supplied, and with the BNF.
- ALL USERS MUST BE ABLE TO RECOGNISE ERYTHRODERMIC AND PUSTULAR PSORIASIS and must know that both are emergencies rather than a reason to supply a stronger cream. See Appendix 1.
- All users must be able to distinguish stable plaque psoriasis from guttate psoriasis, and to recognise secondary infection of a plaque.
- All users must be able to estimate body surface area using the patient's palm as roughly 1%, and to apply the 10% limit.
- All users must understand that stopping a potent topical steroid abruptly can precipitate generalised pustular psoriasis, and must be able to explain the step-down to the patient.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.
- All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.
- All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.
- All users must be up to date with their CPD requirements and appraisal.
- All users must understand capacity and consent, including the Mental Capacity Act 2005. This PGD covers adults only.

The PGD

Indication	Topical treatment of stable plaque psoriasis vulgaris amenable to topical therapy, in adults aged 18 years and over, where systemic therapy is not required and a fixed-dose combination topical treatment is appropriate.
Inclusion criteria	• Adults aged 18 years or older. • Stable plaque psoriasis, diagnosed or previously diagnosed, mild to moderate, amenable to topical therapy. • **Affected area 10% of body surface or less. Use the patient's palm as roughly 1%.**

	● **Lesions on the trunk, limbs or scalp. Not the face, genitals or flexures.** ● **No exclusion criterion present, and no red flag from Appendix 1.** ● Informed consent given. ● **FIRST 4 WEEK COURSE, or a further course where the GP has reviewed and agreed continuation. See the repeat supply row.**
Exclusion criteria: the form of psoriasis	**These are SPC contraindications AND emergencies. Refer the same day, and do not supply.** ● **ERYTHRODERMIC PSORIASIS: widespread redness affecting most of the skin, with scaling, often with shivering, feeling unwell, or being unable to keep warm. This is a medical emergency; patients can lose fluid, heat and protein and become septic.** ● **PUSTULAR PSORIASIS: sheets or clusters of small sterile pustules on red skin, whether localised to palms and soles or generalised. Generalised pustular psoriasis is a medical emergency.** ● **EXFOLIATIVE psoriasis, meaning widespread peeling or shedding of the skin.** ● **GUTTATE psoriasis: multiple small drop-like lesions, often after a sore throat. There is no experience with this product in guttate psoriasis. Refer.** ● Unstable or rapidly worsening psoriasis of any kind.
Exclusion criteria: all other	**Refer, do not supply, where any of the following applies.** ● Under 18 years of age. Safety and efficacy have not been established below 18. ● Hypersensitivity to calcipotriol, to betamethasone or any corticosteroid, or to any excipient of the formulation supplied. ● **KNOWN DISORDER OF CALCIUM METABOLISM.** ● **More than 10% of body surface affected (extensive disease on NICE's definition: refer for specialist advice), or the patient would need more than 15g in a day.** ● **Facial, genital or flexural psoriasis. Refer; the skin at those sites is very sensitive to corticosteroids.** ● **Viral skin lesions including herpes simplex and varicella; fungal or bacterial skin infection; parasitic skin infection; skin manifestations of tuberculosis or syphilis.** ● Perioral dermatitis, rosacea, acne rosacea or acne vulgaris. ● **Atrophic skin, striae atrophicae, fragility of skin veins, or ichthyosis.** ● Ulcers or wounds in or near the area to be treated. ● **Secondary infection of the plaques. Treat the infection first, under the appropriate PGD or by referral; if infection worsens, the corticosteroid must be stopped.** ● **Currently using any other topical corticosteroid on the same area. Concurrent treatment with other steroids on the same site must be avoided.** ● Current phototherapy, or systemic immunosuppressive or systemic anti-psoriatic treatment. ● **Pregnant, planning pregnancy, or breastfeeding.** ● Severe renal impairment or severe hepatic disease. Safety and efficacy have not been evaluated. ● **Already had a 4 week course under this PGD without GP review since. See repeat supply.**

	● Informed consent not given.
Repeat supply, and why it is capped	**Repeat supply with no limit on how many times is indefinite supply of a potent topical corticosteroid with no review, which is not what a PGD is for.** **Under this version:** ● ONE 4 week course may be supplied under this PGD. ● **A further course requires that the GP has reviewed the patient since the last course and agreed continuation. Ask, and record the answer and the date.** ● **MAXIMUM THREE 4 WEEK COURSES IN ANY 12 MONTHS under this PGD, whatever the GP has said. Beyond that, refer.** ● **The SPC is explicit: continuing or restarting after 4 weeks should be under medical review and regular medical supervision. A pharmacy supply is not medical supervision.** **Psoriasis is a lifelong condition. This service treats a flare; it does not manage the disease.**
Stopping the treatment: rebound	● **Stopping a potent topical corticosteroid abruptly in psoriasis can precipitate GENERALISED PUSTULAR PSORIASIS, or a rebound flare worse than the original.** ● The SPC records a case of a patient who used far above the maximum dose for 5 months, developed Cushing syndrome, and then developed pustular psoriasis after abruptly stopping. **What to tell every patient:** ● Do not simply stop at 4 weeks and do nothing. Continue emollients, and see your GP as arranged. ● **Seek medical advice the same day if, after stopping, the skin becomes widely red and sore, starts shedding, or develops crops of small pustules.** ● **Do not restart the cream yourself to control a rebound. Come back or see the GP.**
Cautions	**Do not use under occlusive dressings. Occlusion increases systemic absorption of the corticosteroid.** Avoid application to large areas of damaged skin, mucous membranes or skin folds, for the same reason. **Wash hands thoroughly after each application to avoid transfer to the face, mouth and eyes.** Do not shower or bathe immediately after application. **Advise the patient to limit or avoid excessive natural or artificial sunlight during treatment. Calcipotriol may enhance the effect of UV radiation.** **VISUAL DISTURBANCE. Blurred vision or other visual symptoms have been reported with topical as well as systemic corticosteroids. Refer to an ophthalmologist for evaluation: possible causes include cataract, glaucoma and central serous chorioretinopathy.** **Hypercalcaemia can occur if the 15g daily maximum is exceeded. Symptoms include increased urination, constipation, muscle weakness and confusion. It resolves on stopping.** Diabetes: systemic absorption of a potent corticosteroid can affect glycaemic control. Advise closer monitoring during the course. The ointment contains butylhydroxytoluene (E321), which may cause contact dermatitis or irritation of the eyes and mucous membranes. **Long-term use increases the risk of local effects (skin atrophy, telangiectasia, striae, folliculitis, depigmentation) and systemic effects (adrenal suppression, cataract, raised intraocular pressure). This is why**

	the course is capped.
Actions if the patient is excluded or declines	Explain why supply is not appropriate and what to do instead. Where the presentation is erythrodermic, pustular or exfoliative, make the urgency explicit and arrange same-day assessment rather than a routine appointment; those patients need to be seen today. Where the exclusion is extent, area or a repeat beyond the cap, explain that the issue is the limits of this service rather than the treatment being wrong for them, and refer to the GP. Advise emollients in every case, which are appropriate at any severity and need no PGD. Document the advice given and the decision reached.

The medicine

Name, form and strength	• Calcipotriol 50 micrograms/g with betamethasone (as dipropionate) 0.5 mg/g. • OINTMENT: for body and limb plaques. Examples: Dovobet ointment, and generic equivalents. • GEL: licensed for scalp as well as body. Examples: Dovobet gel, and generic equivalents. • CUTANEOUS FOAM: Enstilar. • CREAM: Wynzora. • **CHECK THE FORMULATION IS LICENSED FOR THE SITE BEING TREATED before supplying. The strength is the same across all four; the licensed sites are not.** • **Xamiol gel is discontinued and must not be ordered.**
Legal category	POM. A potent (group III) corticosteroid; the label will state strong steroid.
Dose and frequency	Apply once daily to affected areas. MAXIMUM 15g IN ANY ONE DAY. Body surface treated must not exceed 10% (the PGD ceiling; the licence limit is 30%).
Quantity to be supplied	Up to 100g per week, within the 15g daily maximum and appropriate to the area treated. Supply enough for the 4 week course and no more.
Maximum treatment period	4 WEEKS. One course under this PGD. A further course only where the GP has reviewed since, and a maximum of three courses in any 12 months.
Route and method of administration	Topical, to affected skin only. Allow the preparation to remain on the skin or scalp; do not shower or bathe immediately after applying. Wash hands thoroughly after each application. Do not use under occlusive dressings.
Storage	Store below 25 degrees Celsius. Do not refrigerate. Keep the container tightly closed. Follow the SPC for the formulation supplied; after first opening the ointment has a 1 year shelf life.
Drug interactions	No interaction studies have been performed. The material point is concurrent topical treatment: do not use another topical corticosteroid on the same area, and do not combine with other topical anti-psoriatic products, systemic anti-psoriatic treatment or phototherapy, for which there is limited or no experience.
Adverse effects	Common: skin exfoliation, pruritus. Uncommon: skin atrophy, exacerbation of psoriasis, dermatitis, erythema, rash, purpura or ecchymosis, burning sensation, skin irritation, folliculitis, skin infection, application site pigmentation change and pain. Rare: PUSTULAR PSORIASIS, REBOUND EFFECT, hypercalcaemia, striae, photosensitivity, acne, dry skin, furuncle, hypersensitivity. Not known: blurred vision. Systemic corticosteroid effects including adrenal suppression, cataract and raised intraocular pressure can occur, more frequently with occlusion, large areas or long-term treatment.

Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given. • Patient name, address, date of birth, and the GP with whom they are registered. • **THE FORM OF PSORIASIS, recorded as stable plaque, and that erythrodermic, pustular, exfoliative and guttate presentations were considered and excluded.** • **BODY SURFACE AREA AFFECTED, as a percentage, and how it was estimated.** • Sites treated, and confirmation that the face, genitals and flexures are not involved. • **Which formulation was supplied and that it is licensed for the site treated.** • Name and brand of the product, formulation, pack size, batch number and expiry date, quantity supplied, and the date. • **WHETHER THIS IS A FIRST COURSE OR A REPEAT. Where a repeat, the date of the GP review that agreed continuation, and the number of courses in the last 12 months.** • **That the rebound advice was given, in terms.** • Name and registration number of the healthcare professional supplying. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Keep for 8 years.
Disposal	Advise the patient to return any unused product to a pharmacy.

Information for the patient

Written information	Supply the patient information leaflet for the formulation given, together with written advice on how much to use, the 4 week limit, and what to do when the course finishes.
Counselling	• **Once a day, to the patchy areas only. Not on your face, genitals or in skin folds.** • **No more than 15 g in a day, which is about half of a 30 g tube, and not on more than about a third of your body. Your palm is roughly 1% of your skin.** • **Wash your hands thoroughly afterwards, so none of it reaches your face or eyes.** • Do not cover the treated area with a dressing or wrap. • Do not shower or bathe straight after putting it on. • Keep using your emollients. They are the foundation and you should not stop them. • **FIRE RISK FROM EMOLLIENTS AND OINTMENTS. Emollients and paraffin-based ointments, including paraffin-free products, soak into clothing, bedding and dressings and make them catch fire more easily and burn faster. Tell the patient not to smoke, use a naked flame, or go near anything burning, and to wash clothing and bedding often, knowing that washing may not remove the residue completely.**

	• Avoid a lot of sun or sunbeds while you are using this. • **THIS IS A 4 WEEK COURSE. Do not keep using it beyond that without seeing your GP.**
When the course ends, and when to get help	**Stopping matters as much as starting.** • **When you finish, keep using emollients and see your GP as arranged. Do not just stop everything.** • **GET MEDICAL ADVICE THE SAME DAY if your skin becomes widely red and sore, starts shedding or peeling all over, or you develop crops of small pus-filled spots. That can happen after stopping a strong steroid and needs seeing quickly.** • **Do not restart the cream yourself to control that. Come back to us or see your GP.** • **Also seek advice for blurred vision or other eye symptoms, and for increased thirst, needing to pass urine often, constipation, muscle weakness or confusion.**

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions.
- Psoriasis: assessment and management, NICE clinical guideline CG153, published 24 October 2012, last updated 1 September 2017, with the Summaries of Product Characteristics for Dovobet 50 micrograms/g + 0.5 mg/g ointment and Enstilar 50 micrograms/g + 0.5 mg/g cutaneous foam (LEO Laboratories Limited, both revised 6 September 2024). Summarised 9 September 2026.
- The current Summary of Product Characteristics for each product named in this PGD, medicines.org.uk.
- British National Formulary, current edition.

Authorisation of this version

Authorised by the Medical Director and the Head Pharmacist named below. This version is not valid without both signatures.

Version	v008
Supersedes	Version 007, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

PGD adoption and authorisation by the employer or clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service, and that appropriate governance is in place. This section must be signed by the superintendent or clinical lead responsible for this service.

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
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.....	..... | ..... | ..... | .....
Valid from date	30 September 2026
Expiry date	31 July 2027

Change history to version 003

Later versions are recorded in the Version and change record at the end of this document.

Version	v003
Date	9 September 2026
Changes	● The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored, together with the standard governance requirements: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function that affected 15 live documents. Raised as blocking by an adopting pharmacy. ● The fire risk warning for emollients and paraffin-based ointments is restored to the counselling. Emollients, including paraffin-free ones, soak into fabric and make it burn faster. Version 001 carried the warning and version 002 lost it. Found by comparing this document against the archived version 001, not by anyone noticing. ● Erythrodermic, exfoliative and pustular psoriasis added as absolute exclusions and identified as same-day emergencies, with Appendix 1 describing how to recognise each. All three are SPC contraindications and none was mentioned anywhere in version 001. ● Guttate psoriasis excluded, there being no experience with this product in that form. ● Rebound on discontinuation addressed for the first time. Stopping a potent topical corticosteroid in psoriasis can precipitate generalised pustular psoriasis, and version 001 said nothing about it to the pharmacist or the patient. Counselling and a same-day referral trigger are now required. ● Repeat supply capped. Version 001 permitted repeats "where previous response was appropriate" with no limit, which is indefinite supply of a potent steroid without review. Now one 4 week course, a further course only where the GP has reviewed since, and a maximum of three courses in any 12 months. ● Known disorders of calcium metabolism added as an exclusion, an SPC contraindication that was absent. ● The SPC contraindication list completed: parasitic skin infection, skin manifestations of tuberculosis or syphilis, perioral dermatitis, atrophic skin, striae atrophicae, fragility of skin veins, ichthyosis, and ulcers or wounds. Version 001 listed only viral, fungal and bacterial infections, rosacea and acne. ● Concurrent use of another topical corticosteroid on the same area added as an exclusion. ● 30% body surface limit moved from the dosing note into the inclusion and exclusion criteria, with Appendix 2 on estimating it by palm. ● Maximum 15g in any one day stated as the operative limit, this being the SPC figure that protects against hypercalcaemia. ● Formulations set out individually with a requirement to check the formulation is licensed for the site treated, and a note that Xamiol gel is discontinued. ● Visual disturbance added, with referral to an ophthalmologist for possible cataract, glaucoma or central serous chorioretinopathy. ● Secondary infection of plaques handled: treat the infection first, and stop the corticosteroid if infection worsens. ● Records must now carry the form of psoriasis with the emergency presentations expressly excluded, the body surface percentage and

	how it was estimated, which formulation and whether it is licensed for the site, whether this is a first or repeat course with the GP review date, and that the rebound advice was given.

Previous versions

001	Development and issue of new PGD. Calcipotriol with betamethasone, adults 18 and over, mild to moderate plaque psoriasis. The product and strength were correct; the contraindications, the rebound risk and the limit on repeat supply were not.

Appendix 1: The three presentations that are emergencies, not flares

All three are SPC contraindications and all three need same-day assessment. None appeared anywhere in version 001.

ERYTHRODERMIC psoriasis	• Widespread redness over most of the body, with scaling and shedding. • The patient often feels cold, shivery or unwell, and may be tachycardic. • Fluid, heat and protein loss, and risk of sepsis and cardiac failure. • **SAME-DAY EMERGENCY REFERRAL. Do not supply.**
GENERALISED PUSTULAR psoriasis	• Crops or sheets of small sterile pustules on red, tender skin. • Often with fever and feeling unwell. • **Can be precipitated by stopping a potent topical steroid.** • **SAME-DAY EMERGENCY REFERRAL. Do not supply.**
EXFOLIATIVE psoriasis	• Widespread peeling or shedding of skin in sheets. • **SAME-DAY REFERRAL. Do not supply.**
GUTTATE psoriasis (not an emergency, but not for this PGD)	• Many small drop-shaped lesions, often on the trunk, frequently following a streptococcal sore throat. • There is no experience with this product in guttate psoriasis. Refer.
Localised pustular psoriasis of palms and soles	• Pustules confined to palms and soles. • Still a contraindication to this product. Refer, though not as an emergency unless the patient is unwell.

Appendix 2: Estimating body surface area

- The patient's whole palm including the fingers is roughly 1% of their body surface.
- 10% is roughly ten palms, about one arm's worth of skin (rule of nines: one arm is about 9%). In practice, if psoriasis covers more than about one arm's worth of skin, refer.
- **Record the estimate as a percentage, not as "extensive" or "limited".**
- The 10% ceiling is NICE's threshold for extensive psoriasis, where topical treatment alone is unlikely to give satisfactory control and specialist referral should be considered. The calcipotriol licence limit of 30% (hypercalcaemia risk) still applies to any use and is never reached under this PGD.

Appendix 3: Key references

- Summary of Product Characteristics for Dovobet ointment, Dovobet gel, Enstilar cutaneous foam and Wynzora cream, and their generic equivalents, current versions.
- NICE CKS: Psoriasis.
- British National Formulary.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, https://www.nice.org.uk/guidance/mpg2

Version and change record
Version: v008
Issued: 30 September 2026
Supersedes: Version 007, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v007
Issued: 14 September 2026
Supersedes: Version 006, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v006
Issued: 11 September 2026
Supersedes: Version 005, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Decision 32: the body surface ceiling is lowered from 30% to 10% (NICE's threshold for extensive disease), with referral above it; the guidance summary, the SmPC summary, the training requirement, the inclusion and exclusion criteria, the dose row, the counselling and Appendix 2 all state 10%, and the 30% calcipotriol licence limit is described as a licence limit that is never reached under this PGD.
Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.
Previous version records
Version: v005
Issued: 11 September 2026
Supersedes: Version 004, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Guidance summary now distinguishes NICE's 10% referral consideration from the PGD's 30% licence ceiling.
Records row now requires formulation, pack size, batch number and expiry date, as the other topical PGDs do.
Duplicate training bullets (indemnity, capacity and consent) removed, and the consent bullet no longer refers to children and young people in an adult-only PGD.
Counselling row replaced '15g tube-worth' (no 15 g pack exists) with 15 g a day, about half of a 30 g tube.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v004
Issued: 11 September 2026
Supersedes: Version 003, 8 September 2026

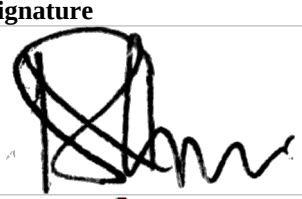
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v008, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.